جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : عبير محمد مهدي ابراهيم الحويطي
المحافظة : القليوبيه **الحي / المدينة :** القناطر الخيريه **العنوان :** 00
رقم البطاقة : 28712121404422 **رقم التليفون :** 01062260390 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 12-12-1987 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: عبير محمد مهدي ابراهيم الحويطي
National ID: 28712121404422

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٠١١٥٢٠٠٢٧٤٣



National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستاف روسي انترناشونال مصر

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM CC / E / 2026 G

Name	عبير محمد موسى	MRN	٦٠٩٤
		ID	
Age	٣٨ سنه	Phone	
Menopausal status	Pre	Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

Clinical Data

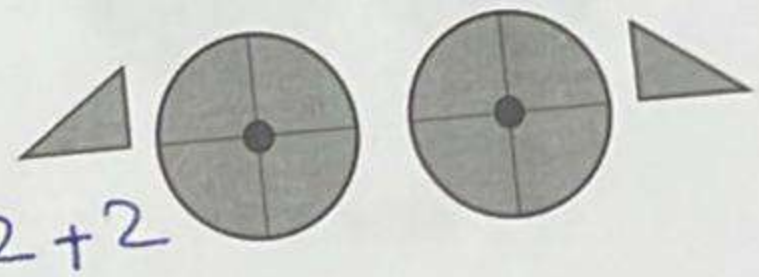
BX:

ER:	PR:	HER 2:	Ki67:

Sonomammography:
2021 → ER 7/8 PR 8/8 her2 +2
R+ MRM

Mets Workup: Adj : PoRTH + HT
2024 → HFL
Ⓑ BX : ER 3/8 - PR 0/8 - her2 -ve

R L



TNM

Recommendations:
For : Xeloda Single agent
+ Zoladex 10.8.13m.

MDT OUTCOME & PLAN
① Re-do Mast w/l +/- biopsy for newly detected contralateral Cancer Br.
② Monthly shot of Zoladex (+) meno w/u.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
22/1/22					

Name: عبير محمد مهدى	
MR. : 692	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

Premenopausal patient with locally recurrent breast cancer, supraclavicular and subpectoral metastases to mediastinal lymph nodes, ER3/8, PR HER2-negative. Initial management included taxol and carboplatin chemotherapy, followed by tamoxifen. The patient had a history of radiotherapy and mastectomy in 2021 and is currently on zoledronic acid for osteopenia.

The patient, 39 years old, was diagnosed with right breast IDC in 2021 (cT4b N2, ER7/8, PR8/8, HER2 2+ not amplified). She received neoadjuvant chemotherapy and a right mastectomy in 2021, followed by a PORT and endocrine therapy with zoladex/AI. In 2024 she developed a locoregional recurrence and possible HFLs; a biopsy revealed metastatic breast cancer ER3/8, PR0/8, HER2-negative. She was started on taxol/carboplatin, completing therapy in November 2024 with stable disease. She reports increasing size of neck nodes; examination shows a 2 × 2 cm right supraclavicular node. A CT of the chest, abdomen, and pelvis (CAP) on 27 January 2026 shows a regressive course. In view of discrepancy between clinical and radiological findings, short-term clinical assessment of response is planned while continuing tamoxifen.

The patient reports an episode of vaginal bleeding lasting 5 days, which resolved with medical treatment. A vaginal ultrasound on 2 March 2026 shows a 5 mm endometrial thickness. Examination reveals a 2 × 2 cm right supraclavicular node that appears stationary. Follow-up includes TVUS and a repeat CT CAP; tamoxifen is continued.

The patient presents with a de-novo metastatic breast cancer, low ER, BRCA-negative, and is under maintenance tamoxifen. She reports increasing neck mass; examination shows a 2 × 2 cm right supraclavicular node and a 1 × 1 cm left supraclavicular node. An urgent CT neck and CAP are ordered, followed by multidisciplinary team review.

Labs were accepted for xeloda and supportive care. A trial to resume zoladex through MOH supportive approval was requested. A cervical lymph node biopsy dated 30 April 2026 revealed IDC grade 3, ER 7/8 (60 %), PR 0/8, HER2 +2 (not amplified).

The patient reports neck swelling; examination shows a 1.5 × 1 cm right supraclavicular node and a 2 × 2 cm lateral clavicle node. The disease is stable. Plans include C3 after labs, CT CAP, and bone scan assessment.

The patient reports generalized bony pain; examination shows a stationary right supraclavicular node and a clavicular nodule. Bone scan is performed. Labs are normal. The patient is started on C4 xeloda after labs.

The patient reports persistent generalized bony pain; examination shows a stationary right supraclavicular node and a clavicular nodule. Labs are within normal limits. The patient is started on C5 xeloda and will bring CT scan results at the next follow-up.

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	

الطبيب المعالج : شيماء فاروق يحيى لاشين

يعتمد ,

A CT CAP performed on 19 July 2026 shows a stationary course with resolution of the HFL. Labs are within acceptable values. The patient is started on C6 xeloda today. The patient has completed 6 cycles of Xeloda.

GUSTAVE ROUSSY

Patient Name	: الأستاذه / عبير محمد مهدى ابراهيم الحريطى	Request No.	: 13006210
Patient No.	: 28712121404422	Request Date	: 30/04/2026
Path No.	: 1251/26	Verification Date	: 12/05/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Known case of breast cancer with suspicious cervical lymph node
Nature of Specimen: Tru-cut biopsy, collected at 30/04/2026, 12:40 pm, cold ischemia time: 20 seconds.

Gross Examination:
- Multiple grayish white tissue cores largest 0.5 cm, totally submitted.

Microscopic Examination:
- Hisotpathological examination of left cervical lymph node showed metastatic deposits of invasive duct carcinoma grade 3 with positive lymphovascular emboli (in capsule).

Diagnosis:
Left cervical lymph node, Tru-cut biopsy:
Positive for metastatic invasive duct carcinoma, Grade 3.
For further IHC (GATA-3, ER, PR and Her2).

Signature
Dr. Walaa Ibrahim Ghoneim

Verified by :
Dr Maha Guimie

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

USTAVE/
OUSSY
ERNATIONAL

Patient Name	: الأستاذه / عبير محمد مهدى ابراهيم الحويطى	Request No.	: 13006210
Patient No.	: 28712121404422	Request Date	: 30/04/2026
Path No.	: 1251/26	Verification Date	: 12/05/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left cervical lymph node "diagnosed as metastatic invasive duct carcinoma grade 3".
Nature of Specimen: Tru-biopsy, collected at 30/04/2026, at 12:40 pm.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- GATA-3 clone (L50-823), ER clone (1E2), PR clone (SP1), Her2 clone (4B5).

Results:
* IPT (All controls showed appropriate reactivity):
- GATA-3: Diffuse strong positive.
- ER: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- PR: No nuclear reaction in tumor cells (score 0/8).
- Her2: Moderate complete membrane reaction in more than 10% of tumor cells (score 2+).

Conclusion:
Left cervical lymph node, Tru-cut biopsy:
IPT:
- ER: Positive (score 7/8).
- PR: Negative (score 0/8).
- Her2: Equivocal (score 2+). For SISH.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and

Verified by :
Dr Maha Guimie

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Immunohistochemistry
[[DOCTYPE:IHC]]

USTAVE OUSSY
ERNATIONAL

Patient Name	الأستاذه / عبير محمد مهدى ابراهيم الحويطى	Request No.	13006210
Patient No.	28712121404422	Request Date	30/04/2026
Path No.	1251/26	Verification Date	12/05/2026
Sex	Female	Financial Category	جوستاف روسي - نفقة الدولة
Age	38 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghoneim

Verified by :
Dr Maha Guimie

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Immunohistochemistry
[[DOCTYPE:IHC]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / عبير محمد مهدى ابراهيم الحويطى	Request No.	: 13006210
Patient No.	: 28712121404422	Request Date	: 30/04/2026
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Age	: 38 Y	Department	: Out Patient
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Pathology Report

Clinical Data: Left cervical lymph node "diagnosed as metastatic invasive duct carcinoma grade 3".
Nature of Specimen: Tru-biopsy, collected at 30/04/2026, at 12:40 pm.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- Her2 clone (4B5), & SISH.

Results:
* IPT (All controls showed appropriate reactivity):
- Her2: Moderate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.
(Average Her2 copy No.: (2.05), Her2/CEP17 ratio: (0.93)

Conclusion:
Left breast cancer, Tru-cut biopsy:
IPT:
- Her2: Equivocal (score 2+).
- SISH: Not amplified (Group V).

* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and

Verified by :
Dr Maha Guimie

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY

Patient Name	:الأستاذه / عبير محمد مهدى ابراهيم الحويطى	Request No.	: 13006210
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Age	: 38 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Dr. Walaa Ibrahim Ghoneim

Verified by :
Dr Maha Guimie

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient's Name:	**Abeer Mohammed Mahdi Ibrahim**
Date:	**2026-07-19**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**



MULTISLICE CT EXAMINATIUON OF THE NECK ,CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal course and calibre of the carotid and jugular vessels.
- Normal CT appearance of the submandibular and parotid salivary glands showing homogenous enhancement. No focal solid or cystic lesions.
- No appreciable changes seen as regards to the previously noted bilateral deep cervical LNs and the largest on the right side still measures about 14mm.
- Evident right mastectomy with no evidences of either residual/recurrent neoplastic lesions seen at the operative bed.
- Normal left breast and both axillae.
- Stability of the previously-noted left perivascular lymph nodes with no other sizable mediastinal lymph nodes noted.
- Hyperinflated both lungs.
- Stability of the mild subpleural reticulations seen in the anterior segment of right upper pulmonary lobe and a thin linear atelectatic band seen in the left lower pulmonary lobe.
- No sizable newly-developed abnormally-enhancing pulmonary nodules , patchy areas of consolidations or foci of calcifications noted.
- No other sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Normal bones of the thoracic cage.
- Mildly increased sized liver,measuring about 17.3 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- Vanished the previously-seen tiny non-enhancing lesion in segment IVb.
- No sizable newly-developed abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 11.4 cm. No focal lesions seen inside.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	**Abeer Mohammed Mahdi Ibrahim**
Date:	**2026-07-19**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

- Normal CT appearance of the pancreas, functioning both kidneys(**with persistence of right lower caliceal stone,measuring about 9.2mm with CT density of 1000HUs**) , both suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.
- No appreciable changes as regards to the tiny sclerotic bony lesions previously-seen in the right iliac and to lesser extent in the left pubic bone.

IMPRESSIONS:

A status follow up since the lastly done study at 19/04/2026 with the current study shows:

- **Evident right mastectomy with no evidences of either residual/recurrent neoplastic lesions seen at the operative bed.**
- **Vanished the previously-seen tiny non-enhancing lesion in segment 1Vb of the left hepatic lobe,denoting a complete resolusion.**
- **No appreciable changes seen as regards to the previously noted bilateral deep cervical LNs and the largest on the right side still measures about 14mm,denoting a stable course.**
- **Stability of the previously-noted left perivascular mediatinal lymph nodes, the mild subpleural reticulations in the anterior segment of right upper pulmonary lobe and a thin linear atelectatic band in the left lower pulmonary lobe and no appreciable changes as regards to the tiny sclerotic bony lesions previously-seen in the right iliac and to lesser extent in the left pubic bone,denoting their stable course.**

Prof/Hanan El-Ahwal.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	**Abeer Mohammed Mahdi Ibrahim**
Date:	**2026-07-19**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Hyperinflated both lungs.
- No sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Mosaic appearance of the scanned bones with osteolysis/sclerotic foci scattering in the scanned spine with wedge collapse of DV8 and to lesser extent DV11,and DV12.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	**Abeer Mohammed Mahdi Ibrahim**
Date:	**2026-07-19**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Mildly enlarged liver,measuring about 17.5 cm, showing a homogenous parenchymal attenuationand e

CT Scan
[[DOCTYPE:CT_SCAN]]

Name : Abeer Mohammed Mahdi Ibrahim
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 22-06-2026 06:07
Medical No. : 399388

Tc-99m MDP Whole-Body Bone Scintigraphy

Clinical History & Indication

A 37-year-old female patient with a history of right breast cancer presented for routine follow-up staging and skeletal surveillance. Active complaints of specific bony aches are noted.

Technique

Following the intravenous administration of Tc-99m MDP via an indwelling venous access port (Port-A-Cath), anterior and posterior whole-body planar scans were acquired approximately 2 hours post-injection.

Findings

- Symmetric and physiological tracer distribution is demonstrated throughout the axial and appendicular skeleton. No abnormal focal zones of increased or decreased osteoblastic activity are identified to suggest active metastatic bone disease.
- Mild, non-specific tracer uptake is noted along the spine and major joints, consistent with age-appropriate degenerative changes.
- Normal physiological excretion is visualized through the kidneys and urinary bladder.
- There is a focal area of intense tracer accumulation noted in the left infraclavicular region on the anterior projection. This strictly correlates with the documented site of Port-A-Cath tracer injection and represents local reservoir retention/extravasation. No underlying structural osseous abnormality corresponds to this site.

Impression

- **Negative Bone Scan:** The skeleton demonstrates normal tracer distribution with no scintigraphic evidence of active or cold focal osseous lesions to suggest metastatic bone disease.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

جوستاف زوسي
انترناشونال
مصر

إسم المريض ثلاثي: .....
رقم الملف: .....

تقريـــر طبـــى

السيد/.....

تحية طيبة وبعد،،

إيماء للطلب المقدم من ..... بشـان تقرير طبـي للمـريض: عبير سعد عطوان

بقسم.....رقم طبي .....

نتشرف بأن نوضح فيما يلي تقريرا عن حالته :

تاريخ وساعة الدخول..... .....تاريخ وساعة الخروج.....

وبعد الكشف والفحوصات والمتابعة وجد انه:

.....

.....وبها علامات تشنج لاشكال البلع

.....الكبار من والعود

.....Xalade

.....

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تحريرا في

الوقت: ..... **التاريخ :** ٢٠/٩/٢٠٢٣

الطبيب المعالج

.....

يعتمد ،،

جوستاف زوسي
انترناشونال
مصر

إسم المريض ثلاثي: .....
رقم الملف: .....

تقريـــر طبـــى

السيد/.....

تحية طيبة وبعد،،

إيماء للطلب المقدم من ..... بشـان تقرير طبـي للمـريض: عبير سعد عطوان

بقسم.....رقم طبي .....

نتشرف بأن نوضح فيما يلي تقريرا عن حالته :

تاريخ وساعة الدخول..... .....تاريخ وساعة الخروج.....

وبعد الكشف والفحوصات والمتابعة وجد انه:

.....

.....وبها علامات تشنج لاشكال البلع

.....الكبار من والعود

.....Xalade

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تحريرا في

الوقت: ..... **التاريخ :** ٢٠/٩/٢٠٢٣

الطبيب المعالج

.....

يعتمد ،،